New York Rehab Care PT PC

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Patient Summary

CASE #1070

Last Name:	DUMERVIL	First Name:	PANDORA	Middle Name:	M

SSN:	105-98-4368	Address:	261 LENOX RD	Primary Phone:	(646) 289-1355
Date Of Birth:	05/02/1994	Apt:	2C	Secondary Phone:	---
Age:	32 Years Old	City:	BROOKLYN	Email:	---
Sex:	Female	State:	NY		
Marital Status:	Single	Zip:	11226		

POLICY HOLDER

First Name:	PANDORA	Date Of Birth:	05/02/1994	Primary Phone:	(646) 289-1355
Last Name:	DUMERVIL	Age:	32 Years Old	Secondary Phone:	---
		Sex:	Female	Email:	---

INSURANCE (PRIMARY)

Ins Name:	Integon National Insurance Company	Case Type:	No Fault	Phone:	(800) 848-1777
Address:	P.O BOX 2989	Role:	Driver	Fax:	---
City:	Clinton	D.O.A:	06/06/2026	Email:	---
State:	IA	Policy #:	65156930		
Zip:	52733	Claim #:	260623831		

PERSONAL INJURY FIRM INFORMATION

Firm:	Robert J. Renna, P.C.	Phone:	(718) 422-1003
Address:	50 Court St	Fax:	---
City:	Brooklyn	Email:	robert.renna@r-ennalaw.com
State:	NY		
Zip:	11201		

NM WELLNESS IN FAMILY HEALTH NP PLLC (2220 Flatbush Ave, Brooklyn NY, 11234 (11))

PATIENT: DUMERVIL, PANDORA	**DOB:** 05/02/1994	**SEX:** Female
DOCTOR: MATIAS NATARA, MONIQUE	**VISIT:** 09/23/2026	**DOA:** 06/06/2026

Initial Visit

Accident type: MVA

Pandora Dumervil is a 32 year old female born on 05/02/1994 . In general, the patient considers herself to be in good health.
Pandora Dumervil was involved in an automobile collision on Saturday, June 6, 2026 involving 2 vehicle(s). The collision occurred in Brooklyn (on Vanderbilt). The patient stated that she was struck from the left side. The patient informed me that at the time of the collision she was a driver. Pandora also informed me that she did not loose consciousness during the accident. During the collision, the patient's head hit the side window.
Other: Chest hit Steering Wheel Furthermore, Pandora states that she was wearing a shoulder strap seat belt.
After the accident, Pandora went to CUNY DownState- walked to hospital Hospital, where she was treated and released the same day. The patient had X-Rays taken over her right knee and lower back.

Complaints

Pain in Head: 7 out of 10. Aggravated by Physical Activity (ADLs). Alleviated by Advil. Type of pain: Sharp,Aching.
Pain in Lower back: 5 out of 10. Aggravated by Bending back,Bending down,General Movement,Getting up from seating position,Going down the stairs,Going up the stairs,Lumbar rotation,Physical Activity (ADLs). Alleviated by Clinical Treatment,Laying Down. Type of pain: Sharp,Aching,Stiffness.
Pain in Right knee: 5 out of 10. Aggravated by Bending down,General Movement,Going down the stairs. Alleviated by Clinical Treatment. Type of pain: Cramping,Aching.
Other complaints: Headaches.

History

Medical and Mental Health History

Allergic to None or nka.
Developmental Concerns: None reported
Medical History: NONE
Surgical History: Right Knee August 27, 2026
Hospitalization History: NONE
Psychiatric/Psychological History: None reported
Diagnostic Studies/Medical Records: none
Previous Accidents: Patient denied having any previous accidents

Current Medication

Non-Psych Medications: n/a

Social History

Substances: Patient denied use of any substances
Cannabis: No.
Alcohol: None.
Tobacco: None
Caffeine: No.

Legal History: Patient denied any significant legal history
Marital Status: Single.
Additional information: Patient presents with resiliency and the capacity to adjust and adapt to the medical and psychological effects associated with the reported accident. Patient continues to experience symptoms that may contribute to functional limitations and interfere with activities of daily living. Patient remains engaged in the treatment plan and continues to participate in physical therapy, rehabilitation care, and supportive services as clinically indicated. Continued treatment is considered medically appropriate based on the patient's current clinical presentation, reported symptoms, functional status, and response to treatment. The patient's condition, functional limitations, and progress will continue to be monitored, and the treatment plan will be modified as clinically appropriate. Patient was encouraged to remain compliant with all recommended treatment and follow-up care. Prognosis and disability status will be reassessed based upon clinical progress and response to treatment. Continued supportive and rehabilitative care remains appropriate at this time.

Review of Systems

Positive for Const. (Health in General): Negative

Positive for Ears, Nose, Mouth & Throat: Negative

Positive for C-V (Heart & Blood Vessels): Negative

Positive for Resp.(Lungs & Breathing): Negative

Positive for GI (Stomach & Intestines): Negative

Positive for GU (Kindney & Bladder): Negative

Positive for MS (Muscles, Bones, Joints, Hip, Neck, Knee, Elbow, Ankle, Hand): Joint pain, Aching muscles, Back pain

Positive for Neurologic (Brain & Nerves): Negative

Positive for Const. (Health in General): Negative

Positive for Allergic/Immunologic: Negative

Physical Exam

General

Height: 5 FT | 2 inch
Weight: 106 LB
Dominant Hand: Right

General appearance: Well developed, well nourished, alert and cooperative, and appears to be in no acute distress.
Psychiatric: Oriented to person, place, and time.
Heent: Normocephalic, atraumatic, ENT grossly normal
Head: Normocephalic.
Eyes: PERRL, EOMI. Fundi normal, vision is grossly intact.
Ent: External auditory canals and tympanic membranes clear, no nasal discharge, oral cavity and pharynx no inflammation, swelling, exudate, or lesions.
Neck: Neck supple, non-tender WITH PALPATION, NEGATIVE lymphadenopathy, masses or thyromegaly.
Chest: No deformities, no pain on palpation.
Lungs: Clear to auscultation and percussion without rales, rhonchi, wheezing or diminished breath sounds.
Heart: wnl
Cardiac: Normal S1 and S2. No S3, S4 or murmurs. Rhythm is regular. No carotid bruits.
Abdomen: Positive bowel sounds. Soft, nondistended, nontender. No guarding or rebound. No masses. + on bowel sounds.

Extremities: No peripheral edema, cyanosis or pallor. Extremities are warm and well perfused. Capillary refill is less than 2 seconds.
Skin normal color, texture and turgor with no lesions or eruptions.
Neurological: CN II-XII intact. Strength and sensation symmetric and intact throughout. Reflexes 2+ throughout. Cerebellar testing normal.
Kyphoscoliosis: -
Lumbar Lordosis: Normal
Muscle Spasm: +
Cardiovascular: Denies chest pain, night awakening due to trouble breathings, palpitations.
Endocrine: Denies cold intolerance, appetite changes, hair changes.
Gastrointestinal: Denies abdominal pain, change in bowel habits, constipation, nausea, vomiting, rectal bleeding.
Genitourinary: Denies blood in urine, painful urination, loss of bladder control, urinary retention.
Hematology: Denies easy bruising, enlarged lymph nodes, prolonged bleeding.
Musculoskeletal: Denies muscle wasting. See HPI.

Radiology Results

Image Studies. The Patient presented with results of his imaging studies which showed:

Lumbar

Other: LEFT KNEE 1. JOINT EFFUSION 2. Grade I-II LCL sprain. 3. Heterogeneous, thickened and partially torn ACL. 4. Iliotibial band syndrome with hypertrophy of gerdy's tubercle of the proximal fibula. 5. Narrowing of the femorotibial compartments. 6. Tearing posterior horn of the medial meniscus. Discoid shape lateral meniscus. 7. Quadriceps and patellar tendinosis/tendonitis. Supra and infrapatellar plica. Synovitis of Hoffa's infrapatellar fat pad. LUMBAR 1. Straightened lumbar lordosis. 2. Deep thoracolumbar fascitis. 3. Fibroid type changes of the uterus with a thickened junctional zone. PAP smear of the cervix and pelvic sonography is recommended. 4. L4-L5 dise demonstrates a dise bulge with a central 2-3 mm disc herniation with thecal sac flattening. There are multi-level hypertrophie facet change, ligamentum flavum hypertrophy with lateral recess and neural foraminal narrowing. The visualized nerve roots reveal no evidence of structural abnormality. 5. L5-S1 disc demonstrates a disc bulge with a central 2-3 mm disc herniation with thecal sac flattening. There are multi-level hypertrophic facet change, ligamentum flavum hypertrophy with lateral recess and neural foraminal narrowing. The visualized nerve roots reveal no evidence of structural abnormality. 6. LI-L2 through L3-L4 discs are normal in contour without bulging, herniation or neural impingement.

Extremities MR

Incidental Findings: MRI OF RIGHT KNEE WITHOUT CONTRAST IMPRESSION: 1. Grade I sprain of the anterior cruciate ligament. 2. Grade II meniscal injury involving the body and posterior horn of the medial and lateral menisci, as described above 3. Strip of knee joint effusion with suprapatellar extension and synovial thickening. MRI OF LUMBAR SPINE WITHOUT CONTRAST IMPRESSION: 1. L3-L4: Diffuse disc herniation with bilateral foraminal disc protrusions is seen. Bilateral neural foramina are stenosed. Spinal canal is normal. Foraminal disc protrusions measures about 3.9mm. 2. L4-L5: Diffuse disc herniation with bilateral foraminal disc protrusions is seen. Bilateral neural foramina are stenosed. Spinal canal is normal. Foraminal disc protrusions measures about 3.4mm. 3. L5-S1: Diffuse disc herniation of 2.2mm is seen. Ventral thecal sac is effaced. Bilateral neural foramina and spinal canal are normal MRI CERVICAL SPINE WITHOUT CONTRAST IMPRESSION: 1. C2-C3: Left foraminal disc protrusion is seen. Left neural foramen is effaced. Spinal canal is normal. Left foraminal disc protrusion measures 1.9mm. 2. C3-C4: Right foraminal disc protrusion is seen. Right neural foramen is effaced. Spinal canal is normal. Right foraminal disc protrusion measures 1.7mm. 3. C5-C6: Diffuse disc herniation of 1.3mm is seen. Bilateral neural foramina and spinal canal are normal. 4. C6-C7: Central disc protrusion of 1.4mm is seen. Anterior thecal sac in indented. Bilateral neural foramina, spinal canal is normal.

Range of Motion

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Cervical Spine				
	Degree	% of Normal	Normal	Condition
Flexion	45	90%	50	Without any pain
Extension	55	92%	60	Without any pain
Lateral Left Bending	40	89%	45	Without any pain
Lateral Right Bending	40	89%	45	Without any pain
Lateral Left Flexion	45	100%	45	Without any pain
Lateral Right Flexion	40	89%	45	Without any pain
Left Rotation	75	94%	80	Without any pain
Right Rotation	75	94%	80	Without any pain

Lumbosacral Spine				
	Degree	% of Normal	Normal	Condition
Flexion	80	89%	90	With pain
Extension	20	80%	25	With pain
Lateral Left Rotation	30	67%	45	With pain
Lateral Right Rotation	30	67%	45	With pain
Lateral Left Bending	25	83%	30	With pain
Lateral Right Bending	20	67%	30	With pain

Right Knee				
	Degree	% of Normal	Normal	Condition
Flexion	125	93%	135	With pain

Ortho Strength Exam

Knees

	Left	Right
Flexion	/5	5 /5
Extension	/5	5 /5
Knees other	/5	/5

Orthopedic Tests

Ankles

Special test	
Talar Tilt	All wnl
Anterior Drawer	All wnl
Thompson	All wnl
Point Tenderness	All wnl

Knees

Crepitus	Positive right
Point Tenderness Knees	Right medial joint line Right lateral joint line Right suprapatellar

Hips

Special test / Maneuvers	
Trendelenburg Sign	All wnl
Thomas Test	All wnl
Ober's Test	All wnl
Romberg's Test	All wnl
Provocative SI Test	
Faber (Patrik's)	All wnl
Gaelsen	All wnl
Hip Thrust	All wnl
Distraction	All wnl
Compression	All wnl
Gait Patterns	
Trendelenburg	All wnl
Broad Based (ataxia)	All wnl
Shuffling	All wnl
High Stepping (drop foot)	All wnl
Antalgic (reduce stance phase)	All wnl
InToeing (femoral anteversion)	All wnl